PRODUCT OVERVIEW

ZYPREXA
olanzapine · coated tablets

Olanzapine for schizophrenia and bipolar mania

ZYPREXA is indicated for the treatment of schizophrenia and of moderate to severe manic episodes. In patients whose manic episode has responded to olanzapine, it is also indicated for preventing recurrence in bipolar disorder.[1]

Approved indications | Dosing | Safety information | Clinical evidence



> 90%

5-HT2 receptor occupancy at every dose from 5 to 20 mg/day[12]

Mechanism of action

5–20 mg

once-daily dose range, individualised[1]

Dosing

> 2,900

patients in the controlled schizophrenia trials[1]

Clinical evidence

48.6%

vs 24.2%

responders at 3 weeks in manic episodes, vs placebo[11]

Clinical evidence

Approved indications

For adults. Comprehensive clinical background for both conditions is available in Disease background.

01

Schizophrenia

Treatment of schizophrenia. Effective in maintaining clinical improvement during continuation therapy in patients who have shown an initial treatment response.[1]

Disease background

02

Moderate to severe manic episodes

As monotherapy or in combination therapy.[1]

Clinical evidence

03

Prevention of recurrence in bipolar disorder

In patients whose manic episode has responded to olanzapine treatment.[1]

Clinical evidence

Not an approved indication: bipolar depression. This site covers ZYPREXA coated tablets; the injectable formulations have separate prescribing information.[2,3]

Mechanism of action

Receptor binding profile

Olanzapine is an antipsychotic, antimanic and mood-stabilising agent with affinity for several receptor families. It has greater affinity for serotonin 5-HT2 than dopamine D2 receptors.[1]

Across the dose range of 5–20 mg/day, 5-HT2 occupancy was above 90%, while D2 occupancy rose with dose to a mean of 76% at 20 mg/day.[12]

Receptor profile and occupancy →

D1–5	5-HT	H1	M1–5	α1
Dopamine	Serotonin 2A/2C, 3, 6	Histamine	Muscarinic	Adrenergic

Receptor families at which olanzapine showed affinity (Ki < 100 nM) in preclinical studies. Order does not indicate relative affinity.[1]

Dosing at a glance

Administration & dose range

Administered once daily with or without food. The licensed daily dose range is **5–20 mg/day**, individualised according to clinical status.[1]

Dose escalations, when indicated, should occur at intervals of not less than 24 hours to ensure proper clinical assessment.[1]

Consider a **5 mg/day** starting dose in elderly patients (≥ 65 years) or those with renal or moderate hepatic impairment.[1]

Dose range and special populations →

Recommended starting doses

Indication
Schizophrenia
Manic episode, monotherapy
Manic episode, combination therapy
Prevention of recurrence in bipolar disorder

Important safety information

Contraindications

Hypersensitivity to olanzapine or any excipient; known risk of narrow-angle glaucoma.[1]

Dementia-related psychosis

Not recommended: increased mortality and risk of cerebrovascular accident in elderly patients.[1]

Weight and metabolic changes

Monitor weight, blood glucose and lipids at baseline and regularly during treatment.[1]

Neuroleptic malignant syndrome

If signs occur, all antipsychotic medicines, including olanzapine, must be discontinued.[1]

All warnings and monitoring → Adverse reactions →

Clinical evidence at a glance

Schizophrenia

Significantly greater improvement in negative and positive symptoms in 2 of 2 placebo-controlled and 2 of 3 comparator-controlled trials.[1]

Trials →

Bipolar disorder · Manic episodes

Greater reduction in manic symptoms than placebo and divalproex, and as add-on to lithium or valproate.[1]

Trials →

Bipolar disorder · Recurrence prevention

Superior to placebo over 12 months; non-inferior to lithium (recurrence 30.0% vs 38.3%). Not superior as 18-month add-on to lithium or valproate.[1]

Trials →

PRESCRIBING INFORMATION

Consult the prescribing information before prescribing

ZYPREXA is a prescription medicine. Treatment should be initiated and monitored by an appropriately qualified healthcare professional.

View full prescribing information

Next screen: **Disease background** →

Address

CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

+49 7621 593 06-98

info@cheplapharm-reg.com

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DISEASE BACKGROUND

Schizophrenia and bipolar disorder

Clinical background on the two conditions in which olanzapine is used. Product information, evidence and prescribing details are in the sections that follow.

Schizophrenia | Bipolar disorder

Schizophrenia

Schizophrenia is a psychiatric disorder that can affect how a person perceives reality, thinks, experiences emotions and interacts with others. Its presentation varies between individuals, and symptoms can change over time.

Treatment aims to manage symptoms, prevent relapse where possible, support everyday functioning and address the individual's broader healthcare needs.

Understanding the symptom domains

01 Positive symptoms

Experiences or behaviours that are **added** to a person's usual functioning.

- Hallucinations
- Delusions
- Disorganized thinking or speech
- Disorganized behaviour

These symptoms may substantially affect a person's perception of reality and ability to function.

02 Negative symptoms

A **reduction or loss** of aspects of normal functioning. Examples may include:

- Reduced motivation
- Diminished emotional expression
- Reduced speech
- Social withdrawal
- Reduced engagement in everyday activities

Negative symptoms can affect social relationships, occupational functioning and participation in daily life.

03 Cognitive symptoms

Difficulties with the **mental processes** needed for everyday activities. These may include difficulties with:

- Attention and concentration
- Working memory
- Planning and organization
- Decision-making and executive functioning

The pattern and severity of symptoms differ between individuals. Cognitive difficulties should be considered as part of the overall clinical assessment.

Three symptom domains of schizophrenia, with examples. Domains are not mutually exclusive and individual presentations vary.[4]

The neurobiology of schizophrenia

Schizophrenia is associated with complex changes in brain function and neurotransmission. One widely discussed model links three interconnected hypotheses: excess dopamine D_2 activity in the mesolimbic pathway, hypofunction of glutamate NMDA receptors, and hyperfunction of cortical serotonin 5-HT_{2A} receptors.[5]

In this model, NMDA receptor hypofunction and 5-HT_{2A} hyperfunction can each lead, further downstream, to mesolimbic dopamine hyperactivity, which is associated with positive symptoms. Negative and cognitive symptoms are thought to involve other circuits.[5] No single neurotransmitter imbalance fully explains schizophrenia or accounts for every symptom an individual experiences.

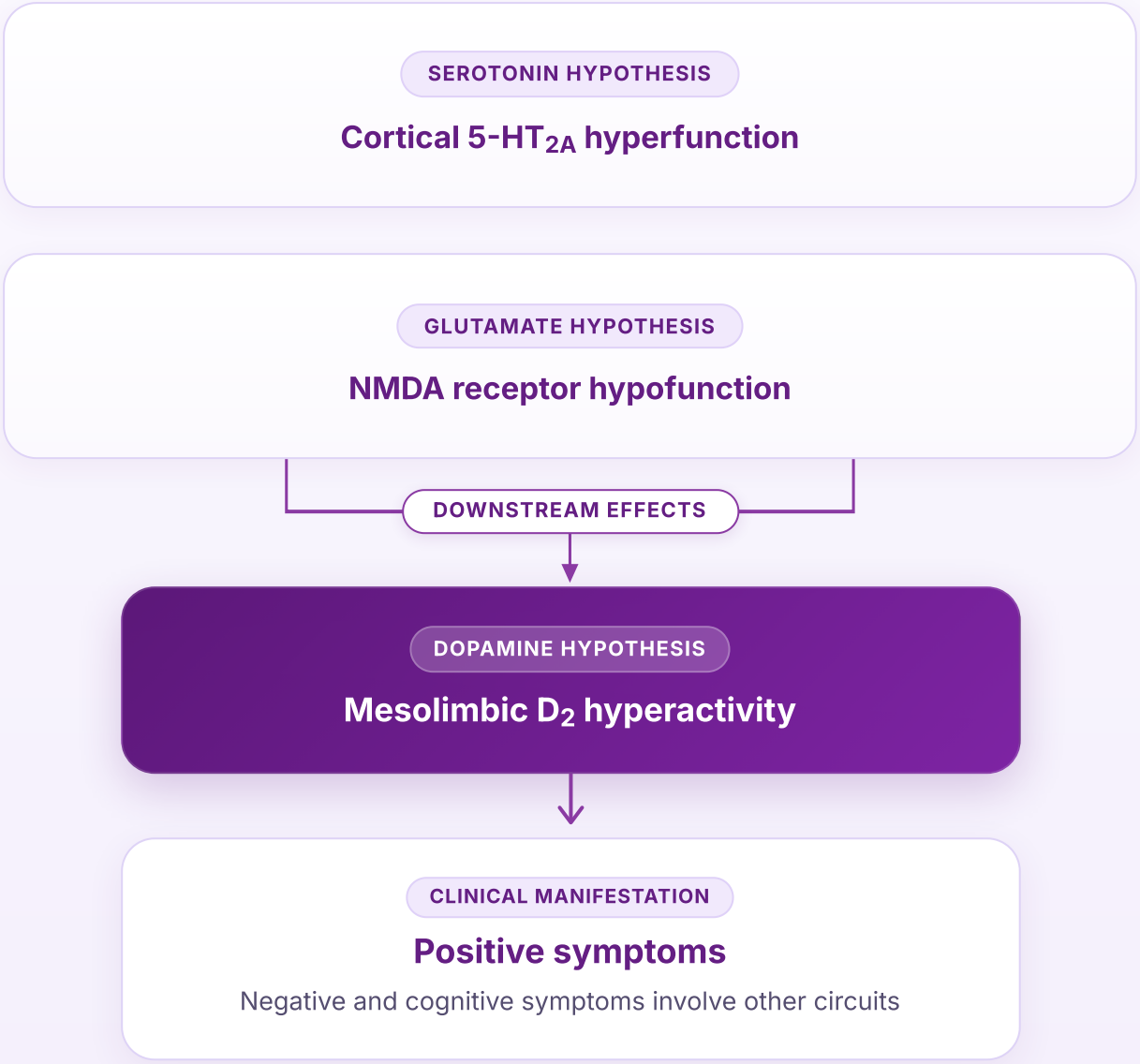
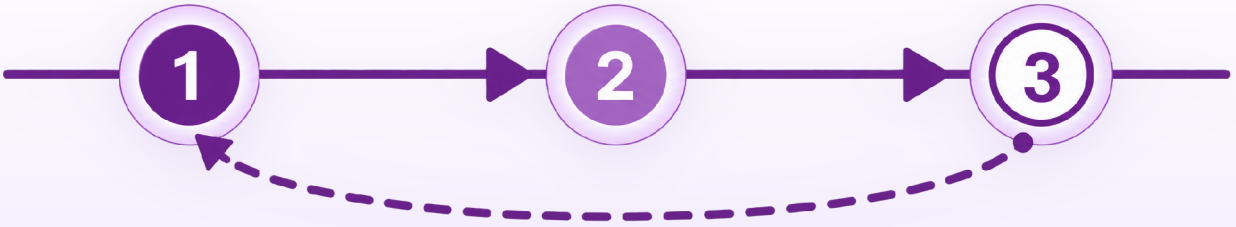


Figure 1. Simplified schematic of the dopamine, glutamate and serotonin hypotheses and their proposed links. It does not depict anatomy or individual neurons. Adapted from Stahl 2018.[5]

Treatment across the course of schizophrenia

Treatment needs can change over time. The following phases are a useful clinical framework rather than a rigid sequence that applies identically to every patient.



01

PHASE

Acute treatment

The immediate objectives are to assess clinical severity and safety, manage acute symptoms and establish an appropriate treatment plan.

02

PHASE

Stabilization

As symptoms improve, treatment focuses on consolidating the response, reviewing tolerability and supporting a return to everyday routines.

03

PHASE

Maintenance

Ongoing treatment and follow-up aim to sustain clinical improvement, monitor adverse effects and identify early signs of deterioration.

Figure 2. Phases of care as a clinical framework. Progression is not fixed, and individual patients may move between phases. Treatment needs can change over time; phases may recur.

Supporting continuity of care

Follow-up should consider treatment response, tolerability, daily functioning, social support and the individual's ability to continue the agreed treatment plan.

Adherence may be affected by adverse effects, the nature of the illness, treatment preferences and practical barriers. Explore these collaboratively rather than attributing them to the patient alone.

Bipolar disorder

Bipolar disorder is characterized by episodes of mood disturbance, including mania or hypomania and depression, depending on the type of bipolar disorder. Episodes can affect energy, sleep, judgement, behaviour and everyday functioning.

Treatment decisions depend on the current episode, previous clinical history, treatment response, comorbidities and the individual's needs.

Understanding mood episodes

ELEVATED, EXPANSIVE OR IRRITABLE MOOD

01 **Mania**

Mania involves a distinct period of abnormally elevated, expansive or irritable mood and increased activity or energy. Symptoms may include reduced need for sleep, increased speech, racing thoughts, inflated self-esteem and impulsive or high-risk behaviour.

Severe manic episodes may be associated with marked functional impairment and can require urgent clinical assessment.

FEATURES OF BOTH

02 **Mixed features**

Some episodes involve symptoms associated with both mania and depression. These presentations can be complex and require careful clinical assessment.

PERSISTENT LOW MOOD

03 **Depression**

Depressive episodes may involve persistent low mood, reduced interest or pleasure, fatigue, changes in sleep or appetite, feelings of worthlessness and difficulties with concentration.

The clinical presentation and treatment needs differ from those of mania.

Features of manic, mixed and depressive episodes.[4] The arrangement groups episode features; it does not represent an illness course, the order in which episodes occur, or their frequency.

Treatment objectives

Treatment objectives depend on the phase of illness.

Acute management

Address the current episode and associated clinical risks.

Stabilization

Consolidate improvement and review tolerability.

Recurrence prevention

Support sustained clinical stability following a treatment response.

These objectives should be individualized. Treatment for an acute manic episode should not automatically be assumed to provide adequate management for every subsequent mood episode.

Product information

Approved indications, dosing and safety information for olanzapine are in the overview and in the dosing and safety sections.

Approved indications →

← Previous screen: **Overview**

Next screen: **Mechanism of action** →

Address

CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

+49 7621 593 06-98

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MECHANISM OF ACTION

Understanding the pharmacology of olanzapine

A broad receptor-binding profile contributes to its pharmacological effects.

Olanzapine is an antipsychotic, antimanic and mood-stabilising agent with activity across several neurotransmitter receptor systems.[1]

Receptor profile | Receptor occupancy | Clinical relevance

Receptor-binding profile

DESCRIBED IN RELATION TO ANTIPSYCHOTIC ACTIVITY

Dopamine receptor activity

D1, D2, D3, D4, D5

Olanzapine has affinity for dopamine receptors, including D2 receptors.[1]

Dopamine receptor antagonism is relevant to its antipsychotic activity. Dopaminergic signalling is involved in multiple brain functions, and the relationship between receptor activity and individual symptom domains is complex.

DESCRIBED IN RELATION TO ANTIPSYCHOTIC ACTIVITY

Serotonin receptor activity

5-HT2A/2C, 5-HT3, 5-HT6

Olanzapine also has affinity for serotonin receptors, including 5-HT2A and 5-HT2C receptors.[1]

Its serotonin and dopamine receptor activity contributes to its overall pharmacological profile. However, the clinical effects of olanzapine cannot be attributed to one receptor interaction alone.

DESCRIBED IN RELATION TO ADVERSE EFFECTS

Histamine H1 receptors

H1

Relevant to effects such as somnolence and increased appetite.[13]

DESCRIBED IN RELATION TO ADVERSE EFFECTS

Muscarinic receptors

M1–M5

Relevant to anticholinergic adverse effects.[13]

DESCRIBED IN RELATION TO ADVERSE EFFECTS

Alpha-1 adrenergic receptors

α1

Relevant to orthostatic hypotension and dizziness.[13]

Receptor families at which olanzapine showed affinity (Ki < 100 nM) in preclinical studies. Relative affinities are not represented. These interactions help explain why treatment selection and monitoring must consider both therapeutic objectives and potential adverse effects.[1]

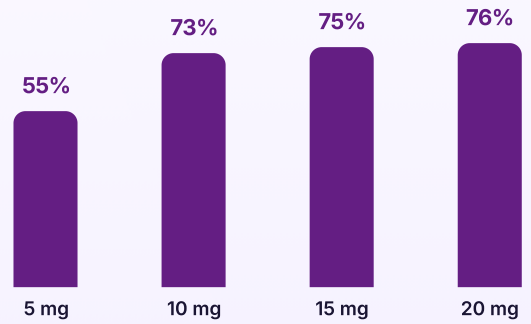
5-HT2 and D2 occupancy across the dose range

Olanzapine has greater affinity for serotonin 5-HT2 than dopamine D2 receptors. In a PET study in patients with schizophrenia, 5-HT2 occupancy was above 90% at every dose, while D2 occupancy rose with dose.[1,12]

Table. Striatal D2 and 5-HT2 occupancy across daily d

Dose	Mean D2	Individual D2 range	5
5 mg/day	**55%**	43–64%	>
10 mg/day	**73%**	71–75%	>
15 mg/day	**75%**	73–76%	>
20 mg/day	**76%**	74–80%	>

Three patients per dose, at steady state.[12]



Mean D2 occupancy by dose. 5-HT2 occupancy was above 90% at every dose from 5 to 20 mg/day (individual values about 91% to 100%). Doses above 20 mg/day reported in the paper are not shown.[12]

Study design and limitations

Twelve patients with schizophrenia were randomly assigned to fixed daily doses of 5–20 mg. Striatal D2 occupancy was measured with [11C]raclopride PET; baseline was estimated from age-corrected controls rather than each patient's own scan.[12]

Receptor occupancy does not by itself predict the clinical response of an individual patient.

Individual occupancy in 12 patients; numbers show mean D2 occupancy per dose. Redrawn from Kapur et al. 1998, Table 1. Doses above 20 mg/day reported in the paper are not shown.[12]

From pharmacology to clinical practice

A mechanism-of-action explanation provides context for understanding the pharmacology of olanzapine. It does not, by itself, establish the magnitude of clinical benefit or predict the response of an individual patient.

Clinical efficacy and safety should be judged on the clinical evidence and the prescribing information.

Review the clinical evidence →

Safety and prescribing

← Previous screen: **Disease background**

Next screen: **Dosing** →

Address

CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

+49 7621 593 06-98

info@cheplapharm-reg.com

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DOSING & ADMINISTRATION

Dosing and administration

Starting doses, dose range and dose adjustment for ZYPREXA coated tablets in adults.[1]

Formulations | Starting doses | Stopping treatment

Formulations

This site covers ZYPREXA coated tablets. The injectable formulations have separate prescribing information.



THIS PAGE

ZYPREXA coated tablets (oral olanzapine)

All dosing and safety content below refers to the oral formulation.



SEPARATE PRESCRIBING INFORMATION

ZYPREXA powder for solution for injection

Short-term intramuscular use for agitation. Different dosing and precautions.[2]



SEPARATE PRESCRIBING INFORMATION

ZYPADHERA (olanzapine pamoate)

Prolonged-release injection with its own dosing and post-injection observation requirements.[3]



SEPARATE ORAL FORMULATION

ZYPREXA Velotab orodispersible tablets

Orodispersible tablets are a separate oral presentation. Dosing and safety on this page refer to ZYPREXA coated tablets. Confirm the Velotab product information before prescribing that formulation.

Starting doses and dose range

Recommended starting doses for oral olanzapine in adults.[1]

Table 1. Recommended starting dose by treatment setting

Approved treatment setting	Starting dose
Schizophrenia	10 mg once daily
Moderate to severe manic episode, monotherapy	15 mg single daily dose
Moderate to severe manic episode, combination therapy	10 mg once daily
Prevention of recurrence in bipolar disorder following response to olanzapine for mania	10 mg once daily

The daily dose range is **5–20 mg**, individualised for each patient. Dose adjustments should be based on clinical assessment. An increase above the recommended starting dose should occur only after appropriate clinical reassessment and generally at intervals of not less than 24 hours.[1]

For patients who have been receiving olanzapine for a manic episode, therapy for preventing recurrence continues at the same dose. If a new manic, mixed or depressive episode occurs, olanzapine treatment should be continued (with dose optimisation as needed), with supplementary therapy to treat mood symptoms, as clinically indicated.[1]

Olanzapine can be given without regard to meals, as absorption is not affected by food.

Special populations

Older patients (65 years and over)

A lower starting dose (5 mg/day) is not routinely indicated but should be considered when clinical factors warrant.[1]

Renal and/or hepatic impairment

A lower starting dose (5 mg) should be considered. In moderate hepatic insufficiency (cirrhosis, Child-Pugh Class A or B), the starting dose should be 5 mg and only increased with caution.[1]

Factors that may slow metabolism

When more than one factor is present (female gender, geriatric age, non-smoking status), consider decreasing the starting dose. Dose escalation should be conservative.[1]

Smokers

Starting dose and range need not be routinely altered. Metabolism may be induced by smoking; clinical monitoring is recommended and a dose increase may be considered if necessary.[1]

Children and adolescents

Not recommended below 18 years of age due to a lack of data on safety and efficacy.[1]

Stopping treatment

Acute symptoms such as sweating, insomnia, tremor, anxiety, nausea or vomiting have been reported rarely when olanzapine is stopped abruptly.[1]

Gradual tapering of the dose should be considered when discontinuing olanzapine. Treatment changes should be managed by the treating healthcare professional.[1]

Before prescribing

Check contraindications, warnings and the monitoring schedule.

Contraindications → **Monitoring schedule →** **Interactions →**

← Previous screen: **Mechanism of action**

Next screen: **Safety & monitoring** →

Address

CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

+49 7621 593 06-98

info@cheplapharm-reg.com

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SAFETY & MONITORING

Safety and monitoring

Contraindications, warnings, monitoring and adverse reactions. This summary does not replace the full prescribing information.[1]

Contraindications | Warnings | Monitoring | Adverse reactions

Contraindications

Other conditions that need caution are covered under warnings and precautions.

Contraindicated

Oral olanzapine is contraindicated in patients with:[1]

- Hypersensitivity to olanzapine or to any of the excipients
- Known risk of narrow-angle glaucoma

Excipients include lactose monohydrate. See the prescribing information for the full list.

Important warnings and precautions

During antipsychotic treatment, improvement in the patient's clinical condition may take several days to some weeks. Patients should be closely monitored during this period.[1]

Dementia-related psychosis and/or behavioural disturbances

Olanzapine is not recommended for patients with dementia-related psychosis and/or behavioural disturbances because of an increase in mortality and the risk of cerebrovascular accident. In placebo-controlled trials (6–12 weeks) in elderly patients (mean age 78 years):[1]

3.5% vs 1.5%

incidence of death, olanzapine vs placebo (2-fold increase)

1.3% vs 0.4%

cerebrovascular adverse events, olanzapine vs placebo (3-fold increase)

The efficacy of olanzapine was not established in these trials.

Key warnings

Metabolic effects

Olanzapine treatment has been associated with weight gain and changes in glucose and lipid parameters. Hyperglycaemia and the development or worsening of diabetes, occasionally associated with ketoacidosis or coma, have been reported uncommonly, including some fatal cases.[1]

Assess relevant baseline risk factors and monitor weight, blood glucose and lipid parameters as set out in the monitoring schedule. See monitoring.

Somnolence and dizziness

Somnolence and dizziness may occur. Patients should be cautioned about operating machinery, including motor vehicles.[1]

Sedation and orthostatic hypotension should be considered when assessing falls risk, concomitant medication and individual tolerability.

Other warnings

Parkinson's disease

Not recommended for dopamine agonist-associated psychosis in Parkinson's disease; worsening of parkinsonian symptoms and hallucinations was reported very commonly.[1]

QT interval

Caution with medicines known to increase QTc, especially in the elderly and in congenital long QT syndrome, congestive heart failure, heart hypertrophy, hypokalaemia or hypomagnesaemia.[1]

Thromboembolism

Venous thromboembolism has been reported uncommonly. Identify risk factors such as immobilisation and take preventive measures.[1]

Seizures

Use cautiously in patients with a history of seizures or factors that may lower the seizure threshold.[1]

Neuroleptic malignant syndrome

Neuroleptic malignant syndrome is a potentially life-threatening condition associated with antipsychotic medicines. Clinical manifestations include hyperpyrexia, muscle rigidity, altered mental status and autonomic instability.[1]

If a patient develops signs and symptoms indicative of NMS, or presents with unexplained high fever without additional clinical manifestations of NMS, **all antipsychotic medicines, including olanzapine, must be discontinued**.

Tardive dyskinesia and other movement disorders

Movement-related adverse effects may occur. The risk of tardive dyskinesia increases with long-term exposure; patients should be assessed for relevant symptoms during treatment.[1]

If signs or symptoms of tardive dyskinesia appear, dose reduction or discontinuation should be considered. These symptoms can temporarily deteriorate or even arise after discontinuation of treatment.

Cardiovascular and orthostatic effects

Orthostatic hypotension and dizziness may occur. Caution is required in patients with relevant cardiovascular risk factors or when medicines with potential interactions are co-administered. Blood pressure should be measured periodically in patients over 65 years.[1]

Sudden cardiac death

Reported post-marketing. In an observational study, the risk with olanzapine was approximately twice that in patients not using antipsychotics, and comparable to pooled atypical antipsychotics.[1]

Anticholinergic activity

Caution in prostatic hypertrophy, paralytic ileus and related conditions.[1]

General CNS activity

Caution with alcohol and other centrally acting medicines. Olanzapine may antagonise the effects of dopamine agonists.[1]

Lactose

Patients with rare hereditary galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicine.[1]

Hepatic and haematological considerations

Transient, asymptomatic elevations of hepatic aminotransferases (ALT, AST) have been seen commonly, especially in early treatment. Follow-up is important in patients with elevated liver enzymes, signs of hepatic impairment, limited hepatic functional reserve or concomitant use of potentially hepatotoxic medicines. If hepatitis is diagnosed, olanzapine treatment should be discontinued.[1]

Caution is also required in patients with low leukocyte or neutrophil counts, medicines known to cause neutropenia, a history of drug-induced bone marrow depression, or hypereosinophilic or myeloproliferative conditions. Neutropenia has been reported commonly when olanzapine and valproate are used concomitantly.

Monitoring during treatment

Example intervals in line with antipsychotic guidelines. Individualise monitoring to the patient's clinical circumstances.[1]

Table 1. Example monitoring schedule

Parameter	Baseline	Week 4	Week 8	Week 12	Thereafter
Body weight	Yes	Yes	Yes	Yes	Quarterly
Blood glucose	Yes			Yes	Annually
Lipids	Yes			Yes	Every 5 years
Blood pressure	Periodically in patients over 65 years				

Patients should be observed for signs and symptoms of hyperglycaemia (such as polydipsia, polyuria, polyphagia and weakness). Patients with diabetes mellitus or with risk factors for diabetes should be monitored regularly for worsening of glucose control.[1]

See the full prescribing information for complete monitoring recommendations.

Weight and metabolic changes over time

The proportion of patients with clinically significant changes in weight, glucose and lipids increased with longer exposure (at least 48 weeks).[1]

64.4%

of adults gained ≥ 7% of baseline weight with long-term exposure (≥ 48 weeks)[1]

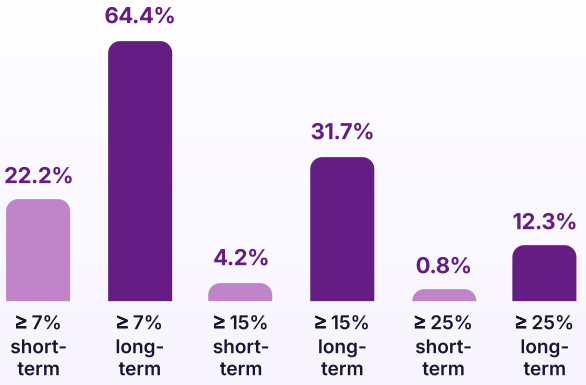
22.2%

gained ≥ 7% with short-term treatment (median 47 days)[1]

39.9%

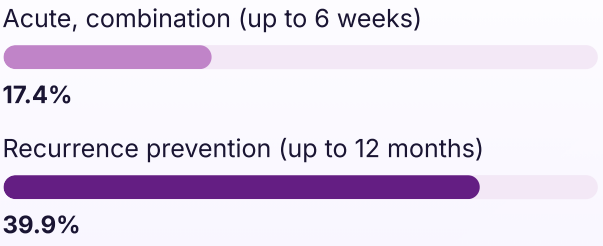
gained ≥ 7% with up to 12 months of recurrence-prevention treatment[1]

All adults, by exposure



Percentage of patients reaching each weight-gain threshold. Clinically significant weight gain was observed across all baseline BMI categories.

Bipolar disorder clinical studies



Patients gaining ≥ 7% of baseline body weight.

Weight-gain thresholds across short-term and long-term exposures and bipolar disorder clinical trials.[1]

Drug interactions

Olanzapine metabolism may be affected by medicines or substances that influence CYP1A2 activity. Review concomitant medicines, smoking status and relevant changes during treatment.[1]

Table 2. Selected interactions

Medicine or factor	Effect	Recommended action
Smoking; carbamazepine (CYP1A2 induction)	May increase olanzapine metabolism and reduce concentrations; only slight to moderate increase in clearance observed	Clinical monitoring; a dose increase may be considered if necessary
Fluvoxamine; other CYP1A2 inhibitors such as ciprofloxacin	Increase olanzapine exposure. With fluvoxamine, mean Cmax rose 54% (female non-smokers) and 77% (male smokers); mean AUC rose 52% and 108%	Consider a lower starting dose; consider a dose decrease if an inhibitor is started
Activated charcoal	Reduces oral bioavailability by 50–60%	Take at least 2 hours before or after olanzapine
Alcohol and other centrally acting medicines	Additive CNS effects	Caution
Direct and indirect dopamine agonists	Olanzapine may antagonise their effects	Anti-Parkinsonian medicines not recommended in patients with Parkinson's disease and dementia
Medicines that increase QTc	Potential additive effect	Caution
Valproate	Neutropenia reported commonly with concomitant use	See hepatic and haematological considerations

Olanzapine does not inhibit the main CYP450 isoenzymes in vitro, and showed no interaction with lithium or biperiden.[1]

Pregnancy and breastfeeding

Because human experience is limited, olanzapine should be used in pregnancy only if the potential benefit justifies the potential risk to the foetus.[1]

Newborns exposed to antipsychotics, including olanzapine, during the third trimester are at risk of adverse reactions including extrapyramidal and/or withdrawal symptoms, and should be monitored carefully.

Olanzapine is excreted in breast milk. Patients should be advised not to breastfeed an infant if they are taking olanzapine.

Adverse reactions

Frequently reported adverse reactions include somnolence, weight gain, increased appetite, dizziness, orthostatic hypotension and changes in glucose and lipid parameters. The full prescribing information gives the complete profile and frequency categories.[1]

Full adverse-reaction table →

~30%

prolactin above normal in trials up to 12 weeks (normal baseline); generally mild[1]

Very common

somnolence and weight gain (≥ 1 in 10 patients)[1]

ALL ADVERSE REACTIONS REPORTED IN ≥ 1% OF ADULTS IN CLINICAL TRIALS

26 reported · Clinical trials

- Somnolence
- Weight gain
- Eosinophilia
- Elevated prolactin
- Elevated cholesterol, glucose and triglyceride levels
- Glucosuria
- Increased appetite
- Dizziness
- Akathisia
- Parkinsonism
- Leukopenia
- Neutropenia
- Dyskinesia
- Orthostatic hypotension
- Anticholinergic effects
- Transient asymptomatic elevations of hepatic aminotransferases
- Rash
- Asthenia
- Fatigue
- Pyrexia
- Arthralgia
- Increased alkaline phosphatase
- High gamma glutamyltransferase
- High uric acid
- High creatine phosphokinase
- Oedema

Frequencies as listed for adults in clinical trials.[1]

PHARMACOVIGILANCE

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via their national reporting system.[1]

PRODUCT INFORMATION

Full prescribing information

The full prescribing information and the patient leaflet for ZYPREXA coated tablets and ZYPREXA powder for solution for injection.
Both links open the European Medicines Agency website.

View full prescribing information

Access patient leaflet

← Previous screen: **Dosing**

Next screen: **Clinical evidence** →

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CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

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CLINICAL EVIDENCE

Clinical evidence for olanzapine

Choose an indication below to see key results. Open a study for its design, safety findings and limitations.

Schizophrenia | Manic episodes | Recurrence prevention | Guidelines

Schizophrenia

Across five controlled trials, olanzapine improved both positive and negative symptoms. A published dose-ranging trial and a secondary analysis of mood symptoms add detail.

2 of 2

placebo-controlled trials: significantly greater improvement in negative and positive symptoms[1]

2 of 3

comparator-controlled trials: significantly greater improvement in negative and positive symptoms[1]

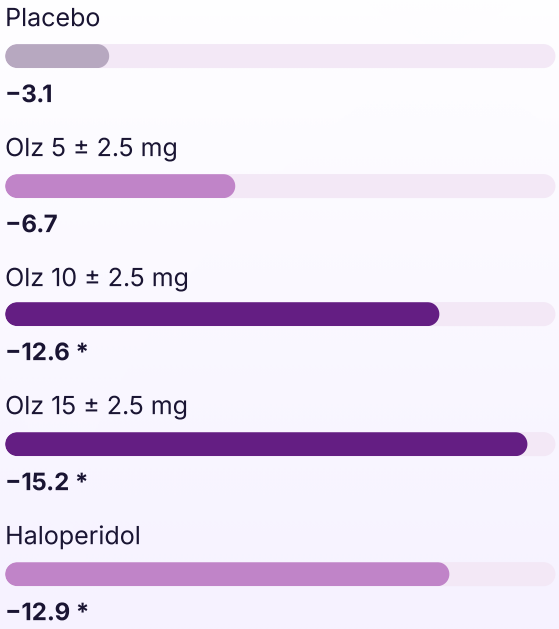
> 2,900

patients across these trials, with both positive and negative symptoms[1]

North American double-blind trial · Beasley et al. 1996 · 6 weeks · n = 335

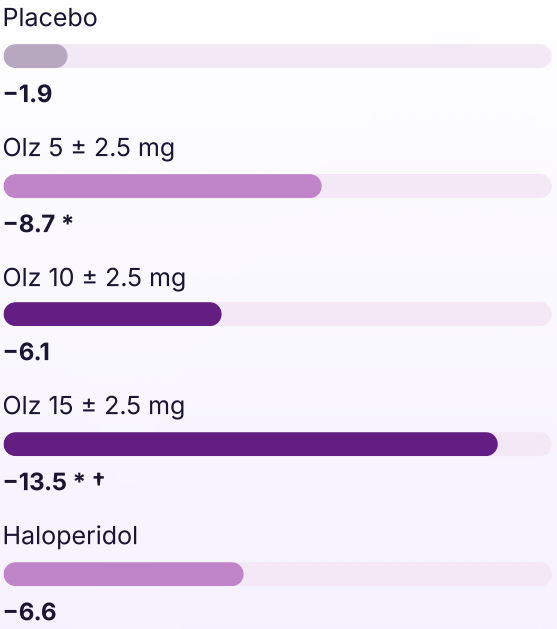
BPRS total −15.2 with olanzapine 15 ± 2.5 mg vs −3.1 with placebo.

RESULTS — BPRS TOTAL (OVERALL SYMPTOMS)



Mean change from baseline (LOCF). * significantly better than placebo.

RESULTS — SANS COMPOSITE (NEGATIVE SYMPTOMS)



* versus placebo. † also versus haloperidol.

In overall symptoms (BPRS total), the medium and high olanzapine dose ranges and haloperidol were superior to placebo.[10]

In negative symptoms (SANS composite), the low and high olanzapine ranges were superior to placebo, and the high range was also superior to haloperidol.[10]

* $p \leq 0.01$ versus placebo ($p \leq 0.05$ for SANS, olanzapine 5 ± 2.5 mg). † $p \leq 0.05$ versus haloperidol. Redrawn from Beasley et al. 1996, Table 5.[10]

STUDY DESIGN

DESIGN	Randomised, double-blind, placebo- and active-controlled; 22 sites, US and Canada
PATIENTS	n = 335; DSM-III-R schizophrenia, acute exacerbation; BPRS total ≥ 24; about 91% chronic course

ARMS	Olanzapine 5 ± 2.5, 10 ± 2.5 or 15 ± 2.5 mg/day; haloperidol 15 ± 5 mg/day; placebo
DURATION	6 weeks (acute phase)
ENDPOINTS	Change in BPRS total and BPRS positive versus placebo; SANS composite versus placebo and haloperidol (LOCF)[10]

SAFETY FINDINGS

39.1%
vs 16.2%

somnolence, olanzapine 15 ± 2.5 mg vs placebo

+3.5 kg

mean weight change at endpoint, olanzapine 15 ± 2.5 mg

0%
vs 13.0%

acute dystonia, olanzapine vs haloperidol

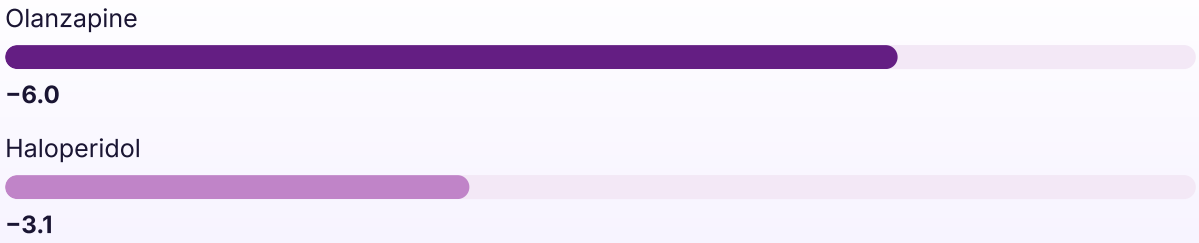
Dose-related transaminase elevations occurred; six olanzapine-treated patients discontinued because of raised ALT without clinical signs of hepatic dysfunction.[10]

LIMITATIONS

- Six-week acute phase only.
- High placebo response.
- The haloperidol dose (mean modal 16.4 mg/day) has been discussed as higher than optimal, which may favour comparisons against it.[10,8]

Mood symptoms in schizophrenia and related disorders · n = 1,481 · baseline MADRS 16.6

MADRS change −6.0 with olanzapine vs −3.1 with haloperidol (p = 0.001).



Mean change from baseline to endpoint. Bars represent improvement as scores fall. p = 0.001.

Patients had schizophrenia, schizoaffective disorder or related disorders, with varying degrees of depressive symptoms. A prospective secondary analysis of mood-score change favoured olanzapine.[1]

Limitation. This was a secondary analysis, not the primary outcome of the study.

Manic episodes

In moderate to severe manic episodes, olanzapine reduced manic symptoms versus placebo and divalproex, and when added to lithium or valproate.

48.6%
vs 24.2%

patients responding at 3 weeks, olanzapine vs placebo (p = 0.004)[11]

−10.3
vs −4.9

mean change in YMRS total at 3 weeks (p = 0.02)[11]

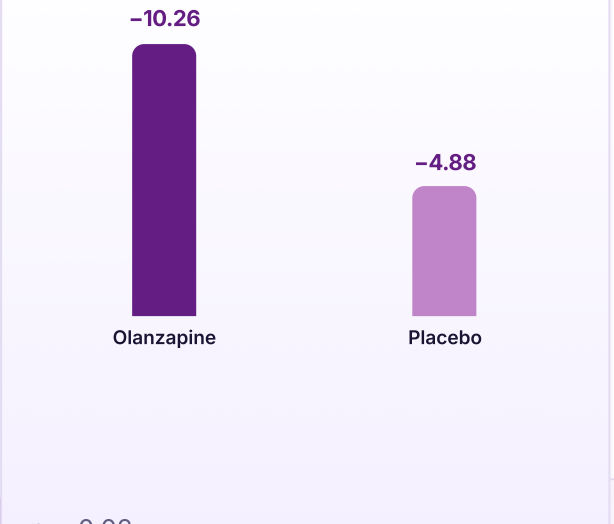
6 weeks

add-on to lithium or valproate: greater reduction in manic symptoms than either alone[1]

Olanzapine versus placebo in acute mania · Tohen et al. 1999 · 3 weeks · n = 139

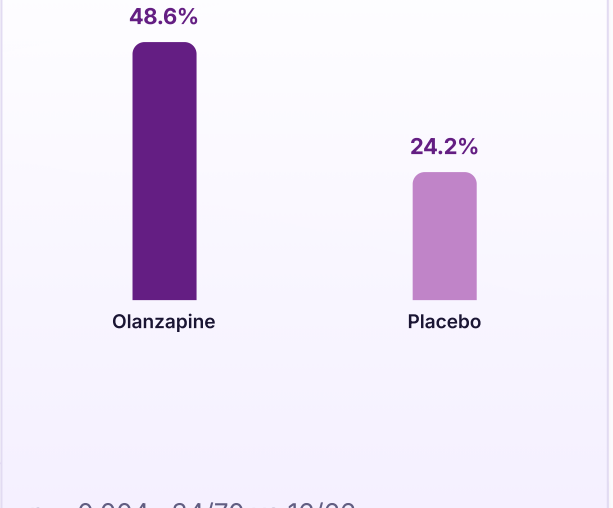
48.6% vs 24.2% of patients responded (p = 0.004).

RESULTS — MEAN CHANGE IN YMRS TOTAL



p = 0.02

RESULTS — RESPONDERS (≥ 50% YMRS DECREASE)



p = 0.004 · 34/70 vs 16/66

patients responded.[11]

Change from baseline to endpoint (LOCF) and proportion of patients with ≥ 50% decrease in YMRS at 3 weeks. Redrawn from Tohen et al. 1999.[11]

STUDY DESIGN

DESIGN	Randomised, double-blind, placebo-controlled, parallel group; 16 US sites
PATIENTS	n = 139 (olanzapine 70, placebo 69); DSM-IV bipolar I disorder, manic or mixed episode; YMRS ≥ 20; 53% with psychotic features
DOSING	Started at 10 mg/day, adjustable 5–20 mg/day; mean modal dose 14.9 mg/day
DURATION	3 weeks
ENDPOINTS	Primary: change in YMRS total (LOCF). Response defined in advance as ≥ 50% YMRS decrease [11]

SAFETY FINDINGS

32.9%
vs 17.4%

somnolence, olanzapine vs placebo

+1.65 kg
vs −0.44

mean weight change over 3 weeks

17.6%
vs 0%

treatment-emergent ALT elevations

Dry mouth (25.7% vs 8.7%), dizziness (22.9% vs 5.8%) and weight gain reported as an adverse event (11.4% vs 1.4%) were also more frequent with olanzapine.[11]

LIMITATIONS

- Three-week duration.
- High placebo dropout (65.2%).
- Concomitant lorazepam permitted.
- The trial started at 10 mg/day; the recommended starting dose for monotherapy of a manic episode is 15 mg/day.[11,1]

Further controlled studies in manic or mixed episodes · 3 to 12 weeks

Versus placebo · 3 weeks. Superior efficacy in reducing manic symptoms.[1]

Versus divalproex · 3 weeks. Superior efficacy to valproate semisodium (divalproex) in reducing manic symptoms.[1]

Versus haloperidol · 6 and 12 weeks. Comparable proportion of patients in symptomatic remission from mania and depression.[1]

Add-on to lithium or valproate · 6 weeks. In patients already treated for at least 2 weeks, adding olanzapine 10 mg reduced manic symptoms more than lithium or valproate alone.[1]

Effect sizes are not available for these studies, so results are described in words.

Recurrence prevention

In patients whose manic episode responded, continued olanzapine was superior to placebo in preventing recurrence and non-inferior to lithium. Added to lithium or valproate, it was not superior to either alone.

Superior

to placebo on bipolar recurrence over 12 months[1]

30.0%
vs 38.3%

recurrence vs lithium over 12 months: non-inferior (p = 0.055)[1]

Not superior

as 18-month co-therapy, versus lithium or valproate alone[1]

Continued olanzapine versus placebo · Remission on olanzapine · 12 months

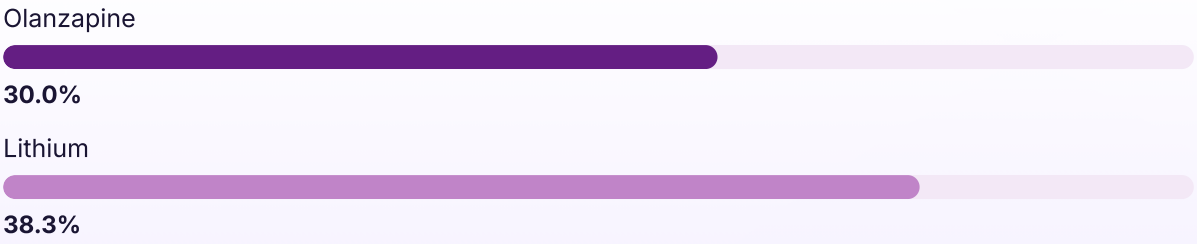
Statistically significantly superior to placebo on bipolar recurrence.

Design. Patients with a manic episode who achieved remission on olanzapine were randomised to olanzapine or placebo for 12 months.

Result. Olanzapine was statistically significantly superior to placebo on the primary endpoint of bipolar recurrence, and also showed an advantage in preventing recurrence into either mania or depression.[1]

Recurrence rates are not available for this study, so no chart is shown.

Olanzapine versus lithium · 12 months · recurrence 30.0% vs 38.3%



Patients with bipolar recurrence at 12 months (primary endpoint). Olanzapine was statistically non-inferior to lithium, p = 0.055.

After remission on olanzapine plus lithium, patients were randomised to olanzapine alone or lithium alone. Olanzapine was statistically non-inferior to lithium on the primary endpoint of bipolar recurrence.[1]

Olanzapine added to lithium or valproate · 18 months

In patients with a manic or mixed episode stabilised on olanzapine plus lithium or valproate, continuing olanzapine co-therapy was not statistically significantly superior to lithium or valproate alone in delaying bipolar recurrence, defined by syndromic (diagnostic) criteria.[1]

Guidelines

Guidelines place olanzapine among several options rather than ahead of them. Recommendations depend on the phase of illness, patient preference and how well the adverse-effect profile suits the individual.

APA practice guideline, 2021[6]

SCHIZOPHRENIA

- Treat with an antipsychotic and monitor effectiveness and side effects (1A).
- Continue antipsychotic treatment after symptoms improve (1A), preferably with the same medicine (2B).
- Clozapine for treatment-resistant schizophrenia (1B).
- Long-acting injectable if preferred or adherence is poor (2B).
- Individual antipsychotics are not ranked.

BAP guidelines, 2016[7]

BIPOLAR DISORDER

- Severe mania: haloperidol, olanzapine, risperidone and quetiapine described as particularly effective.
- Long term: olanzapine is among the agents shown to prevent manic relapse; lithium is recommended as initial monotherapy.
- Weight gain is a prominent adverse effect of olanzapine.

WPA review, 2008[8]

SCHIZOPHRENIA

- Apart from clozapine, no consistent efficacy differences between second-generation antipsychotics.
- Olanzapine is associated with more weight gain and metabolic adverse effects than most comparators.

WHO Essential Medicines List, 2023[9]

PSYCHOTIC AND BIPOLAR DISORDERS

- Olanzapine listed as a therapeutic alternative to risperidone (psychotic disorders) and to quetiapine (bipolar disorders).

Reading this evidence

Consider each result in the context of its population, comparator, endpoint and duration. Individual study results do not predict the response of every patient.

All references →

← Previous screen: **Safety & monitoring**

Next screen: **References** →

Address

CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

+49 7621 593 06-98

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REFERENCES

References

The sources behind the content on this website. Superscript numbers on each page link to the matching entry here.

Literature & documents | Abbreviations

Literature and Product Documents

Search references or keywords...

All References | Prescribing information | Disease background | Efficacy | Mechanism | Guidelines | Safety

1

CHEPLAPHARM Registration GmbH.

ZYPREXA 2.5 mg, 5 mg, 7.5 mg, 10 mg, 15 mg and 20 mg coated tablets. Summary of Product Characteristics. European Medicines Agency..

PRESCRIBING INFORMATION | EFFICACY | SAFETY | MECHANISM

> Primary source for indications, dosing, contraindications, warnings, interactions, adverse reactions, pharmacology and the clinical study programme.
>
> Prescribing information (EMA) ↗

2

CHEPLAPHARM Registration GmbH.

ZYPREXA 10 mg powder for solution for injection. Summary of Product Characteristics. European Medicines Agency..

PRESCRIBING INFORMATION

> Source for the intramuscular formulation.
>
> Prescribing information (EMA) ↗

3

CHEPLAPHARM Registration GmbH.

ZYPADHERA 210 mg, 300 mg and 405 mg powder and solvent for prolonged release suspension for injection. Summary of Product Characteristics. European Medicines Agency..

PRESCRIBING INFORMATION

> Source for the long-acting formulation.
>
> Prescribing information (EMA) ↗

4 **American Psychiatric Association.**

Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision (DSM-5-TR). Washington, DC: American Psychiatric Association Publishing; 2022..

DISEASE BACKGROUND

> Symptom domains of schizophrenia and features of mood episodes.

5 **Stahl SM.**

Beyond the dopamine hypothesis of schizophrenia to three neural networks of psychosis: dopamine, serotonin, and glutamate. CNS Spectr. 2018;23(3):187–191..

DISEASE BACKGROUND MECHANISM

> Review of the dopamine, glutamate and serotonin hypotheses and how they interconnect.
>
> doi:10.1017/S1092852918001013 ↗

6 **Keepers GA, Fochtmann LJ, Anzia JM, et al.**

The American Psychiatric Association Practice Guideline for the Treatment of Patients With Schizophrenia, Third Edition. Washington, DC: American Psychiatric Association; 2021. Executive summary: Am J Psychiatry. 2020;177(9):868–872..

GUIDELINES

> Recommendations on antipsychotic treatment, continuation and long-acting injectables.
>
> Executive summary, doi:10.1176/appi.ajp.2020.177901 ↗

7 **Goodwin GM, Haddad PM, Ferrier IN, et al.**

Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. J Psychopharmacol. 2016;30(6):495–553..

GUIDELINES

> Recommendations for acute mania and long-term treatment of bipolar disorder.
>
> doi:10.1177/0269881116636545 ↗

8 **Tandon R, Belmaker RH, Gattaz WF, et al.**
World Psychiatric Association Pharmacopsychiatry Section statement on comparative effectiveness of antipsychotics in the treatment of schizophrenia. Schizophr Res. 2008;100(1–3):20–38..

GUIDELINES SAFETY

> Review of comparative efficacy and tolerability of antipsychotics.
> doi:10.1016/j.schres.2007.11.033 ↗

9 **World Health Organization.**
The selection and use of essential medicines 2023: report of the WHO Expert Committee (including the 23rd WHO Model List of Essential Medicines). Geneva: WHO; 2024. WHO Technical Report Series, No. 1049..

GUIDELINES

> Lists olanzapine as a therapeutic alternative for psychotic and bipolar disorders.

10 **Beasley CM Jr, Tollefson G, Tran P, et al.**
Olanzapine versus placebo and haloperidol: acute phase results of the North American double-blind olanzapine trial. Neuropsychopharmacology. 1996;14(2):111–123..

EFFICACY SAFETY

> Six-week trial of three olanzapine dose ranges versus haloperidol and placebo in 335 patients with schizophrenia.
> Publisher page ↗

11 **Tohen M, Sanger TM, McElroy SL, et al.**
Olanzapine versus placebo in the treatment of acute mania. Am J Psychiatry. 1999;156(5):702–709..

EFFICACY SAFETY

> Three-week placebo-controlled trial in 139 patients with a manic or mixed episode.
> doi:10.1176/ajp.156.5.702 ↗

12 **Kapur S, Zipursky RB, Remington G, et al.**

5-HT2 and D2 receptor occupancy of olanzapine in schizophrenia: a PET investigation. Am J Psychiatry. 1998;155(7):921–928..

MECHANISM

> PET study in 12 patients with schizophrenia, measuring 5-HT2 and D2 receptor occupancy at doses from 5 to 20 mg/day.
>
> doi:10.1176/ajp.155.7.921 ↗

13 **U.S. Food and Drug Administration.**

ZYPREXA (olanzapine) Prescribing Information. Silver Spring, MD: FDA; 2021..

MECHANISM SAFETY PRESCRIBING INFORMATION

> Clinical pharmacology rationale connecting specific receptor activities (H1, M1, α1) with adverse effect profiles.

Abbreviations

Clinical and pharmacological abbreviations used across this website

Abbreviation	Full clinical definition
ALT, AST	Alanine and aspartate aminotransferase
AUC	Area under the concentration–time curve
BPRS	Brief Psychiatric Rating Scale
CNS	Central nervous system
Cmax	Maximum plasma concentration
CYP1A2	Cytochrome P450 1A2
EMA	European Medicines Agency
Ki	Inhibition constant (binding affinity)
LOCF	Last observation carried forward
MADRS	Montgomery–Åsberg Depression Rating Scale
NMDA	N-methyl-D-aspartate (glutamate receptor)
NMS	Neuroleptic malignant syndrome
PET	Positron emission tomography
QTc	Heart-rate-corrected QT interval
SANS	Scale for the Assessment of Negative Symptoms
SmPC	Summary of Product Characteristics
YMRS	Young Mania Rating Scale

MEDICAL REVIEW STATEMENT

All clinical claims on this platform are referenced to primary literature or current regulatory documents. Superscript numbers correspond to the evidence hub above.

LAST REVIEWED: October 2026

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Address

CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

+49 7621 593 06-98

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Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

+49 7621 593 06-98

info@cheplapharm-reg.com

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CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

Contact

+49 7621 593 06-00

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